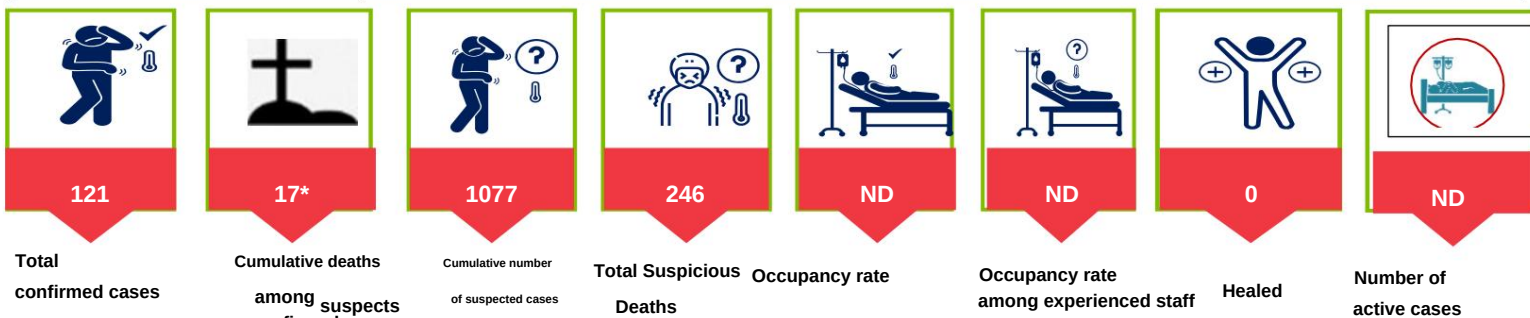


Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC SitRep N°012/MVB_26/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (13)	ITURI : Aru, Bunia, Kilo, Mongbwalu, Nizi, Nyankunde and Rwampara NORTH KIVU : Butembo, Goma, Oicha, Kalunguta and Katwa SOUTH KIVU : Miti-Murhesa
Reporting date	May 26, 2026
Publication date	May 26, 2026



*These deaths are underestimated because confirmation of diagnoses is delayed.

0. PROMINENT POINTS

- Sixteen confirmed cases on May 26;
- Availability offered by UHNAS to PoE for the deployment of countermeasures in all response sites;
- Cumulative confirmed cases May 26, 2026: 121 cases including 110 in Ituri, 10 in North Kivu and 1 in South Kivu;
- Extension assessment:
 - o ITURI: epicenter with 7/36 affected health zones (Aru, Bunia, Kilo, Rwampara, Mongbwalu, Nizi and Nyankunde);
 - o NORTH KIVU: 5/35 ZS affected (Goma, Butembo, Kalunguta, Katwa and Oicha);
 - o SOUTH KIVU: 1/34 ZS affected (Miti-Murhesa);
- 17 confirmed deaths including 10 males (9 aged over 15 and 1 under 15) and 7 females (5 aged over 15 and 2 under 15);
- 6 PPL among the confirmed deaths;
- 1 EDS volunteer stoned by the Community in the Rwampara ZS.

I. CONTEXT

Ituri Province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. It has an estimated population of over 8 million, including more than one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in Ituri province. Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, and experiences frequent population movements towards Uganda. This area is characterized by numerous construction sites that attract people from other territories within the province and from neighboring provinces.

II. HISTORY OF THE EPIDEMIC

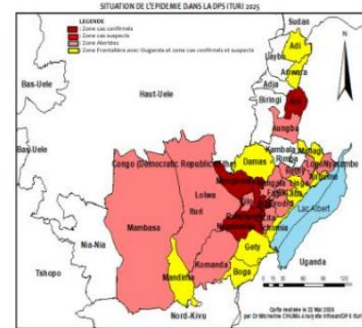
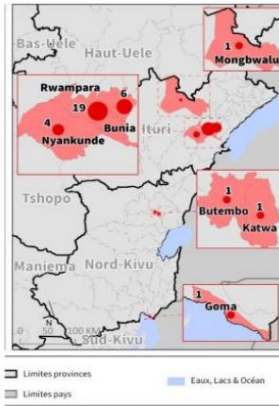
On May 5, 2026, social media reported a surge in deaths of unknown cause, with nearly 50 deaths already recorded in the Mongbwalu health zone. Following the validation of these alerts, the Chief Medical Officer of the Mongbwalu health zone shared the preliminary investigation report on May 9, 2026. On May 11, 2026, provincial health authorities held an emergency meeting and decided to deploy Rapid Intervention Teams (RITs) to Mongbwalu and Rwampara, which had reported suspected deaths of viral hemorrhagic fever among patients admitted to the Rwampara Evangelical Medical Center, who had been transferred from Mongbwalu.

Investigations conducted on May 12, 2026, simultaneously in the Mongbwalu and Rwampara health zones by the Rapid Intervention Teams of the Ituri Provincial Health Division (DPS), revealed that most suspected patients or deaths presented with fever, headache, vomiting, and severe physical asthenia. Deaths occurred 2 to 3 days after the onset of illness, with symptoms including hematemesis, epistaxis, or blackish vomit. During the investigations in the Rwampara health zone, 20 samples were collected from suspected cases or deaths. The majority of samples were taken from the Mongbwalu health zone. Preliminary tests conducted at the Bunia Laboratory were all negative for Ebola Zaire, dengue fever, rotavirus, cholera, malaria, Yersinia pestis, measles, and COVID-19. These samples were sent to the National Institute of Biomedical Research (INRB) in Kinshasa for further testing.

As of May 14, 2026, 13 of these 20 samples were analyzed, and 8 tested positive for non-Zairean Ebola virus disease at the INRB in Kinshasa. On May 15, 2026, sequencing of the positive samples identified the BUNDIBUGYO strain, allowing the Minister of Public Health, Hygiene, and Social Welfare (MSHPS) to officially declare the 17th Ebola virus disease outbreak in the Democratic Republic of Congo.

As of May 17, 2026, in consultation with member states, the WHO Director-General considers that the event taking place in DRC and Uganda meets the criteria of the definition set out in Article 1 of the IHR, and declared that this constitutes a Public Health Emergency of International Concern while specifying that the risk at the global level is low but high at the regional level.

On May 18, 2026, the Executive Director of Africa CDC officially declared a Continental Public Health Emergency (CPHE) under Article 3 of its statutes. This declaration followed an emergency meeting of the Africa CDC Advisory Group, which determined that the worsening outbreak posed an acute risk to continental health security. As a result, the Continental Incident Management Support Team was activated to provide support to affected and at-risk Member States in their response and preparedness efforts.

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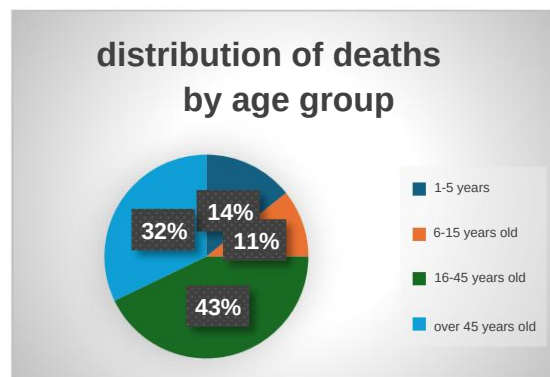
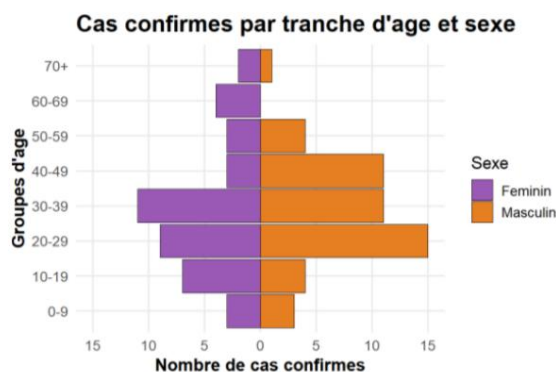
III. RESPONSE ACTIONS IMPLEMENTED

III.1. COORDINATION

- o Holding a meeting at the Prime Minister's office with delegates from the province of Kinshasa, experts from various sectors (Interior, Humanitarian Affairs, Justice, Education, Higher Education) and a delegation of experts from the Presidency
- o Holding a strategic meeting with the Governor of Ituri;
- o Community dialogue around the Mayor of the rural commune of Mongbwalu.

III.2. EPIDEMIOLOGICAL SURVEILLANCE

- o Carrying out field surveillance activities (reporting alerts, investigations, active search, listing and monitoring of contacts, retrospective documentary review of cases and deaths, etc.);
- o Harmonisation of monitoring data with the planning team (database);
- o Finalization of the terms of reference for briefing service providers and community agents in 8 health zones and deployment multidisciplinary teams in health zones.



- The 20–29 age group has the highest number of confirmed cases, especially among men. • The 30–39 and 40–49 age groups are also heavily affected.
- Men generally have more confirmed cases than women in several age groups, particularly 20–29 and 40–49 years old

Point of Entry and Point of Control (PoE/PoC) Surveillance:

- o Validation of the digital notification form for PoE and deployment
- o Pre-deployment briefing of national EIRs on daily reporting;
- o Provision of sprayers to the PoEs of mudziba-Iga barrier and Nizi by the IOM;
- o Supply of three PoCs with water (200 L) by IOM:
- o Supervision of monitoring, PCI and CREC activities in the 5 operational PoCs.

Table I. Summary of PoE/PoC data as of 26/05/2026

VARIABLES	TOTAL
Travelers who have used the PoE/PoC	15,504
Screened travelers	15,504
Travelers who have washed their hands	9,440
Travelers made aware of Ebola virus disease	15,242
Listed contacts	0
Alerts raised	0



Activation of Nizi's PoC

IV.3. LABORATORY

- o Receipt of cartridges in Bunia and continuation of analyses

IV.4. INFECTION PREVENTION & CONTROL

- o Start of training of trainers and support partners on the complete PCI package;
- o Implementation of the 5 EDS carried out at HGR-Bunia, Bunia cité and Ngezi and HGR Rwampara;
- o Disinfection of the multipurpose room and the various offices of the DPS Ituri.



Implementation of the EDS at the Bunia General Referral Hospital



Train-the-trainer and PTF programs on the complete PCI package

III.5. HOLISTIC CARE

- o Continued development of the CTE CME Bunia
- o Holding the technical meeting with all PEC partners;
- o Continued care and clinical evaluation of patients;
- o 2 Deaths including 1 PPL Doctor at the CME and 1 at the HGR Rwampara;
- o Counseling: 12 suspected cases; o
- Emotional debriefing of 45 family members; o Support group:
- 7 families of 2 deceased; o Awareness-raising of 9
- religious leaders and 21 taxi drivers.

III.6. CREC

- o Commitment of 41 traditional healers from the city of Bunia after a briefing on the risks of Ebola, prevention measures and the reporting of alerts;
- o Commitment of the heads of the Bunia markets (central market and Hoho market) and the Mayor of The City of Bunia is taking public health actions by equipping the central market of Bunia with 6 handwashing stations;
- o Awareness-raising activities for 1736 students and teachers from schools in the Rwampara health zones (Espérance Health Center 2 & EP Rwampara) and Aru (EP Agamba, Etoma Liri)



IV.7. LOGISTICS

- o Development of the procedure for authorizing humanitarian flights
- o Supply of PCI inputs and materials to Category 1 ESS facilities: CME/Bunia, HGR Mungwalu and HGR Nyakunde;
- o Reception of inputs from UNFPA;

- o 33 vehicles were made available to the different pillars to ensure their mobility to the axes: Rwampara, Nyakunde, Bunia, Nizi and Bambu; o Start of construction work of the CTE at the HGR Bunia with the support of the IMC.

IV.8. SECURITY

- o Popular demonstrations around 1:30 p.m. at the Nyakunde Evangelical Medical Center (CME) in Bunia in the Q/Hoho in the Bunia Commune against the EDS of the body of Dr Blaise KATHO for the MURONGO/KANDA cemetery.

IV. CHALLENGES

- Contact tracking remains suboptimal, requiring cascading briefings (IS, IT, agents). community) at the level of active and high-risk ZS;
- o Difficulty in mobility for teams at the provincial level and in the ZS (one vehicle per ZS, (no motorcycle)
 - o Delay in delivering results which impacts the small isolation capacity; o Delay in the construction of CTEs in all sites (Nyankunde, HGR Bunia, HGR Rwampara); o Insufficient protective equipment for service providers; o Slow progress in the development of isolation sites (CME, HGR Bunia, Rwampara); o Community resistance to the decontamination of their households and the implementation of EDS.

V. PERSPECTIVES

- o Organize cascading briefings of ZS actors on MVE and SBC surveillance in the affected and high-risk ZS of ITURI province;
- o Continued awareness-raising in public places (markets, schools, churches, etc.); o Development of a Q&A document for community liaisons; o Mapping of all accommodation sites for national experts for security assessment; o Continued decontamination of ESS hotspots and implementation of EDS; o Continuation of multidisciplinary activities across different pillars; o Digitization of the entire community listing and surveillance process; o Launch of field research to reorient the response; o Ensuring a regular supply of lab kits, IPC, etc.; o Etc.

FIELD PHOTOS



Briefing of the Bunia Health Zone decontamination team on the importance of wearing PPE



Implementation of 'EDS' on 2 alert cases



Communication about risks in schools for the benefit of students and teachers: ý s)

**FOR ALL
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